



U.S. Department of Justice
Federal Bureau of Prisons
Field Acquisition Office

*U.S. Armed Forces Reserve Complex
Grand Prairie, Texas 75051*

March 4, 2026

Sources Sought Questionnaire
Comprehensive Medical Services
Metropolitan Detention Center (MDC) Guaynabo
Sources Sought Notice 15BFAO26N00000011

The Federal Bureau of Prisons is conducting market research pursuant to Federal Acquisition Regulation Deviation Part 10. A single solicitation is anticipated to be issued for comprehensive hospital and physician services for inmates at the Metropolitan Detention Center located in Guaynabo, PR (hereinafter referred to as the “institution”). Comprehensive medical services include Inpatient Facility Services, Outpatient Facility Services, Inpatient Physician Services, Outpatient Physician Services, and Outpatient Institution (On-Site Physician) Services. The institution currently houses male and female inmates. Additional information about the inmate population at the institution can be found at www.bop.gov.

This Sources Sought Questionnaire is for information purposes only. The described services may or may not be included in the subsequent solicitation.

Presently, this institution is anticipating estimated quantities for the base and four-one year option periods as follows:

Inpatient and Outpatient Hospital and Physician Services:

Inpatient: 170 Inpatient Days per year (approximately)
Outpatient: 210 Outpatient Visits per year (approximately)

Estimated quantities for other institution services are:

	Visit Duration (approximate)	Visit Quantity per Contract Year (approximate)
General Physician	8	104
General Physician	8	52

	Session Duration Hours (approximate)	Session Quantity per Contract Year (approximate)
Optometrist	8	12

Pricing Methodology – will be as follows: Example: Price proposals shall be calculated from benchmarks utilizing Medicare reimbursement methodologies. For each category of service to be provided, offerors will be allowed to propose a variance from the benchmark Medicare rate in the form of a discount from or a premium to Medicare rates established by the Centers for Medicare and Medicaid Services. **The rates established in the resulting contract shall not be construed as participation in the Medicare program; contract rates will merely be equated to Medicare rates of reimbursement without reductions for deductibles or copayments.** This structuring of the pricing methodology is not intended to be restrictive of any offeror; offerors need only to propose that percentage discount from or premium to the Medicare benchmark rate which will reflect the desired level of payment for the category of services rendered.

Medicare Part A. The Medicare benchmark to be utilized for all services covered by Medicare Part A shall be the most current Basic Medicare Severity Diagnosis Related Group (MS-DRG) payment (i.e., Operating Federal Rate) established for Core-Based Statistical Area (CBSA) 41980-San Juan-Bayamon-Caguas, PR. The Basic MS-DRG calculation shall not include any provider-specific adjustments allowed under actual Medicare participation. Offerors may propose a discount from or a premium to Basic MS-DRG rates.

Medicare Part B Physician Services. The Medicare benchmark to be utilized for all physician services covered by Medicare Part B shall be those rates established in the most current Medicare Part B Fee Schedule for Payment Locality 20, All County Equivalents, PR. The benchmark charges shall be those established for participating providers. Offerors may propose a discount from or a premium to the benchmark charges.

Medicare Part B Facility Services (including hospital outpatient services and ambulatory surgical centers). The Medicare benchmark to be utilized for all facility services covered by Medicare Part B shall be the most current Ambulatory Payment Classifications and Ambulatory Surgical Center rates calculated for CBSA 41980-San Juan-Bayamon-Caguas, PR. Offerors may propose a discount from or a premium to the benchmark charges.

Please answer the following questions:

1. Can your firm provide comprehensive hospital and physician services to inmates at the institution(s) specified above?

_____ Yes _____ No

2. If you answered “No” in Question 1 above, what services can you provide?

3. Please mark the services your firm can provide and whether your firm will be providing services as a prime contractor, through use of a subcontractor, or are unable to provide services. Also, please indicate which services are reimbursable using a Medicare-based pricing structure:

Service Required	Provided as Prime	Provided through a Subcontractor	Unable to Provide	Medicare-Based Pricing
General Physician				Yes or No
Optometrist				Yes or No

4. The North American Industrial Classification System (NAICS) code for General Medical and Surgical Hospitals is 622110, which sets the small business standard at \$47 million.

Based on this standard, what is your business size, small or other than small?

_____ Small _____ Other than Small

5. What level of subcontracting, if any, would be utilized to provide the listed services?

_____ %

6. Are you eligible for participation in one of the following small business programs? If so, please indicate the program.

_____ "8(a)" Program
 _____ Women-Owned Small Business concern
 _____ Economically Disadvantaged Women-Owned Small Business concern
 _____ HUBZone Small Business concern
 _____ Veteran-Owned Small Business concern
 _____ Service-Disabled Veteran-Owned Small Business concern

7. Can your firm provide **Medicare-based prices** for future contract periods (*for example*: a contract with a base year and four option years, equaling a total of five years)? If not, how many years can you provide Medicare-based prices?

8. Please indicate how many years you can provide firm-fixed unit prices and Medicare-based prices.

_____ 5 years _____ 4 years _____ 3 years _____ 2 years _____ 1 year only

9. Can your firm provide centralized billing for all services rendered under the resulting contract? (Definition of centralized billing – The contractor shall provide and operate a system of centralized billing which will enhance the institution's ability to track medical

obligations and expenditures). This system shall, at a minimum, ensure that completed consolidated invoices are submitted to the institution within 90 calendar days after an inmate's discharge or outpatient encounter. Completed CMS-1500 and UB-04 forms shall accompany the contractor's centralized billing invoice for all treatment encounters. The centralized billing invoice shall be limited to no more than fifty (50) CMS-1500 and/or UB-04 forms per invoice. Invoices which are submitted beyond the 90-day requirement shall constitute a performance deficiency under this output and shall be documented in the contractor's performance evaluations. Invoices which are submitted within the acceptable time but are found to contain errors or require further justifications will be returned to the contractor. The contractor shall submit revised/corrected invoices within 30 calendar days from the date of rejection.)

_____ Yes _____ No

(If no, please explain why and what type of billing procedures your firm uses.)

10. What is the "community standard" level of medical liability insurance applicable to:

Hospital Services \$ _____ per occurrence / \$ _____ aggregate

Physician Services \$ _____ per occurrence / \$ _____ aggregate

11. Can your firm prepare a proposal resulting from this solicitation within 30 calendar days?

_____ Yes _____ No

12. If awarded a contract, how many calendar days after the award could you begin providing services? Please note that all individuals providing services at the institutions must undergo a security clearance investigation. This investigation may take 30 calendar days or longer to complete.

_____ Calendar days after the award for Inpatient and Outpatient Hospital services

_____ Calendar days after the award for On-Site Physician Services (Outpatient Institution Services)

13. Please furnish the following information in reference to your firm:

Name:	
Address:	

Point of Contact:	
Telephone Number:	
E-mail Address:	
Unique Entity ID:	

Please e-mail your responses to J2gregory@bop.gov. Responses must be received no later than **2:00 PM EST, March 19, 2026.**

NOTE: The resulting solicitation will be distributed solely through sam.gov. All future information concerning this acquisition, including the pre-solicitation notices, solicitation, and any subsequent amendments, will be distributed solely through the website. Interested parties are responsible for monitoring the website to ensure they have the most current information about the acquisition. If you have questions about SAM, Government procurements in general, or need assistance in the preparation of your proposal, a local Procurement Technical Assistance Center (PTAC) may be able to help. The Procurement Technical Assistance Program was authorized by Congress in 1985 in an effort to expand the number of businesses capable of participating in the Government marketplace. To locate a PTAC near you, go to <http://www.aptac-us.org>.